

Health psychologists interested in studying the influence of positive and negative thinking on weight loss conducted a study on 120 obese adults attempting to lose weight. Participants indicated on a scale of 1 to 7 how successful they thought they would be at losing weight (1 = "not successful" and 7 = "very successful") and how much control they believed they had over their success (1 = "no control" and 7 = "complete control"), recorded as expectation of success scores and control of success scores, respectively.

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At the end of this three-week period, participants began a three-month diet and exercise regimen involving caloric restriction to 1,500 calories per day as well as regular participation in aerobic exercise and weight training. Researchers recorded weekly weights of participants and surveyed participants regarding adherence to proactive weight loss strategies. The results are shown by group in Figures 1 and 2.

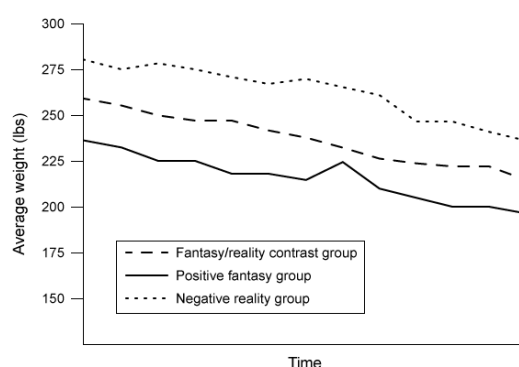


Figure 1 Average weight recorded for each group during three-month follow-up

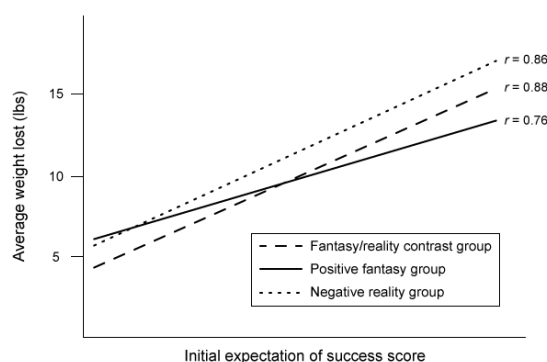


Figure 2 Initial expectation of success score versus average weight loss

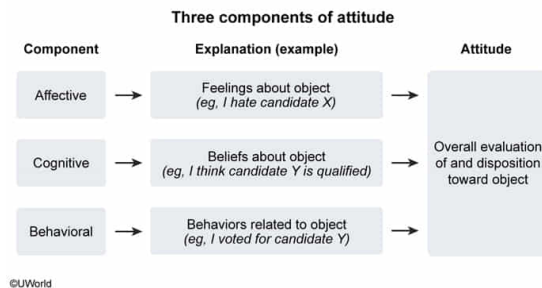
Which of the following study procedures described in the passage is most likely measuring or attempting to evoke the affective component of attitude?

- ✔ ☒ A. Fantasizing about positive aspects of losing weight [57%]
- ☐ B. Reflecting on past failed weight loss attempts [15%]
- ☐ C. Rating predicted success at losing weight [19%]
- ☐ D. Participating in proactive weight loss strategies [7%]

Correct

56% answered correctly

Explanation:



The term **attitude** refers to a person's **evaluation or disposition** toward something (eg, a person, an object, an event). Attitudes can be positive, negative, or neutral and **can change** over time. Psychologists have identified three components of attitudes:

- The **affective** component relates to how a person *feels* about something, including positive or negative evaluations or emotions (eg, anger, excitement). For example, a person might have strong negative feelings about a political candidate, causing her to favor that candidate's opponent.
- The **cognitive** component relates to a person's *beliefs and opinions* about something. For example, a person might believe that a particular candidate has characteristics or experiences that make that candidate well suited to hold office.
- The **behavioral** component relates to how a person *acts* toward something. In the above examples, voting for or against a candidate reflects the behavioral component of attitude.

The study procedure requiring the fantasy/reality contrast condition and the positive fantasy condition to fantasize about the positive aspects of losing weight is most likely attempting to *evoke the affective component of attitude* because positive fantasies will likely generate positive emotions in the participants.

(Choice B) Even though reflecting on past failed weight loss attempts would likely evoke the affective component of attitude, participants *were never* asked to do this as part of the study procedure.

(Choice C) Rating success on a scale of 1 to 7 evokes the cognitive aspect of attitude by forcing the participant to think about a future event. No affect is involved in this task.

(Choice D) Participating in proactive weight loss strategies would be most associated with the *behavioral* (not affective) component of attitude.

Educational objective:

Psychologists outline three components of attitude: affective (feelings), cognitive (thoughts, beliefs, opinions), and behavioral (actions). Attitudes can be positive, negative, or neutral and can change over time.

Time spent:0 sec

Subject:

Behavioral Sciences

QID:400507

System:

Motivation, Emotion, Attitudes, Personality, and Stress

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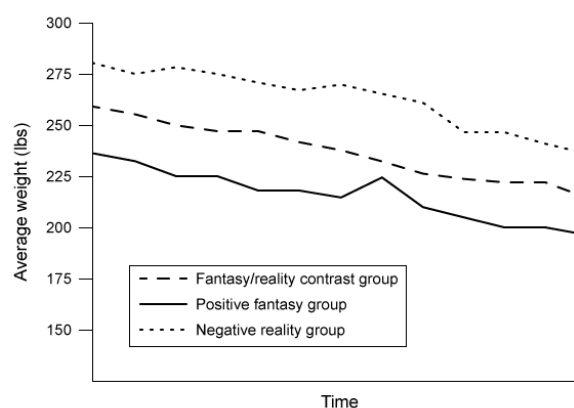


Figure 1 Average weight recorded for each group during three-month follow-up

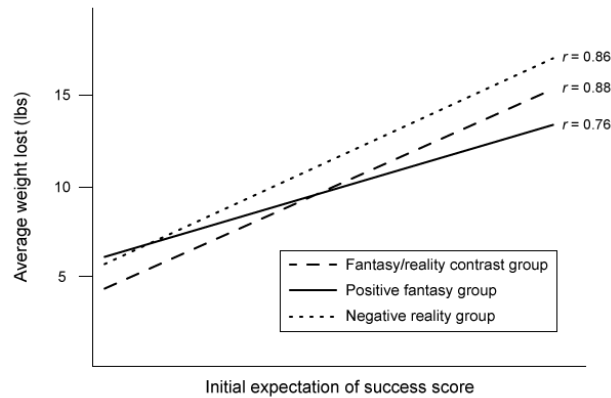


Figure 2 Initial expectation of success score versus average weight loss

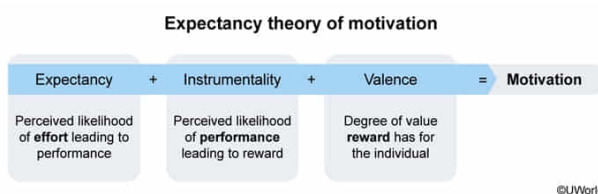
The study design described in the passage is most aligned with which theory of motivation?

- ☐ A. Arousal [7%]
- ✓ ☒ B. Expectancy [61%]
- ☐ C. Incentive [15%]
- ☐ D. Humanistic [15%]

Correct

61% answered correctly

Explanation:



The **expectancy theory of motivation** proposes that individuals are motivated to act based on the **expected outcomes** of their behavior. According to this theory, motivation involves expectancy, instrumentality, and valence:

- **Expectancy** is the belief that one will be *able to achieve the desired outcome*. Asking participants to rate how successful they think they will be at losing weight is a measure of expectancy.
- **Instrumentality** is the belief that one *has control over the desired outcome*. Asking participants to rate how much control they believe they have over their success is a measure of instrumentality.

- **Valence** involves the *value placed on the desired outcome*. If the researchers also asked participants to rate how much they wanted to lose weight, this would be a measure of valence.

(Choice A) The arousal theory of motivation suggests that individuals are motivated to maintain an optimum level of arousal. When underaroused, people seek activities to increase stimulation (eg, if bored at home, one will go out dancing). When overaroused, people seek to decrease stimulation (eg, if overwhelmed at a party, one will step outside for quiet).

(Choice C) The incentive theory of motivation suggests that individuals are motivated according to external rewards. Therefore, people engage in the behavior that results in the most desirable outcomes. For example, people are motivated to go to work to make money.

(Choice D) The humanistic theory of motivation suggests that, assuming more basic needs (like hunger) have been met, individuals are ultimately motivated to seek self-actualization, the ultimate fulfillment of their own potential. The motivation to pursue a medical degree is an example of wanting to fulfill one's potential.

Educational objective:

The expectancy theory of motivation proposes that motivation results from expectancy (believing one is capable of achieving an outcome), instrumentality (believing one has control over the outcome), and valence (believing the outcome is desirable).

Time spent:0 sec

Subject:
Behavioral Sciences

QID:400508

System:
Motivation, Emotion, Attitudes, Personality, and Stress

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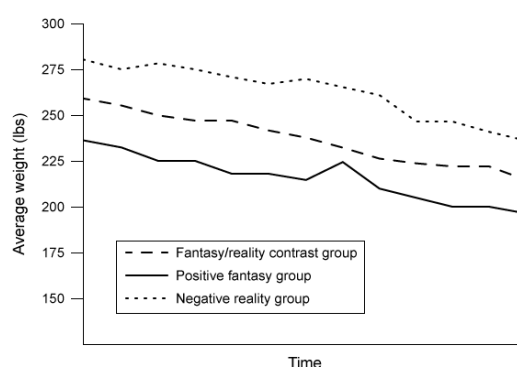


Figure 1 Average weight recorded for each group during three-month follow-up

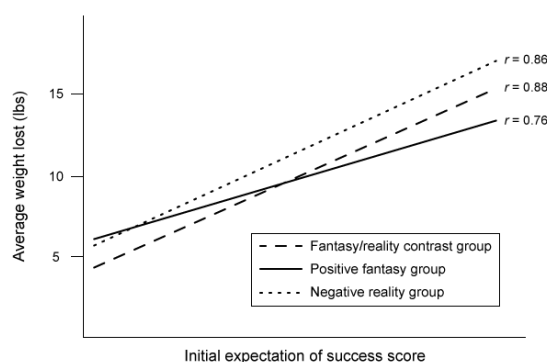


Figure 2 Initial expectation of success score versus average weight loss

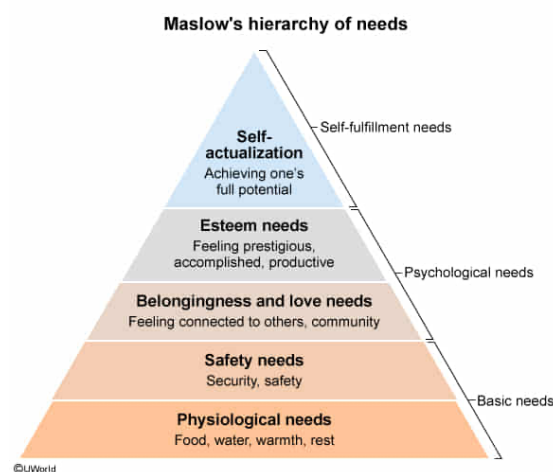
According to the Maslow hierarchy of needs:

- ☐ A. when weight loss is successfully attained, an individual can move on to physiological needs. [3%]
- ✓ ☒ B. a desire to lose weight to look and feel healthy and accomplished is an example of an esteem need. [90%]
- ☐ C. continuing to eat unhealthy comfort food despite obesity fulfills the love/belonging need. [3%]
- ☐ D. losing weight to improve health fulfills the same need as losing weight for social acceptance. [2%]

Correct

89% answered correctly

Explanation:



In the **Maslow hierarchy** of needs, the desire to lose weight to look and feel healthy and accomplished is an example of an esteem need, a psychological need that can be met after basic needs are satisfied. Maslow proposed that humans are motivated to achieve certain needs, which are arranged in a **hierarchy of importance**: basic needs must be met before psychological needs can be addressed, and psychological needs must be met before self-fulfillment needs can be addressed.

- **Basic needs** include physiological needs and safety needs. **Physiological needs** are the basic elements necessary for survival, such as sleep, water, food, and air. **Safety needs** involve protection from danger, a sense of security, law and order, and freedom from fear.
- **Psychological needs** include love/belonging needs and esteem needs. **Love/belonging needs** include the need for connection with friends, family, and romantic partners with regard to affection, trust, and acceptance; they also involve the need to feel integrated into a group, society, or culture. **Esteem needs** involve the desire to feel accomplished and productive within a society. These involve the need for feelings of achievement, independence, and respect.
- **Self-actualization needs** involve a desire to seek personal experiences that allow one to reach full potential. Although Maslow believed that every person was capable of attaining self-actualization, he held that progress toward self-actualization would be thwarted by failure to attain lower needs.

(Choice A) According to Maslow's theory, physiological needs are at the bottom of the pyramid and must be fulfilled before any other needs. Therefore, successful weight loss is not required *before* physiological needs can be met.

(Choice C) A desire to eat food, however unhealthy, most closely corresponds to a physiological need. The desire for love and belonging, as conceptualized by Maslow, pertains to intimate human interactions.

(Choice D) Losing weight to improve health is a basic, physiological need, whereas losing weight for social acceptance is a higher, psychological need.

Educational objective:

The Maslow hierarchy of needs proposes that basic needs (physiological and safety) must be met before psychological needs (love and belonging, esteem) can be attained. All lower needs must be addressed before one can reach the pinnacle of the hierarchy: self-actualization.

Time spent:0 sec

Subject:

Behavioral Sciences

QID:400509

System:

Motivation, Emotion, Attitudes, Personality, and Stress

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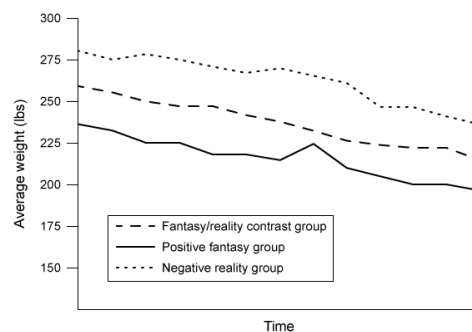


Figure 1 Average weight recorded for each group during three-month follow-up

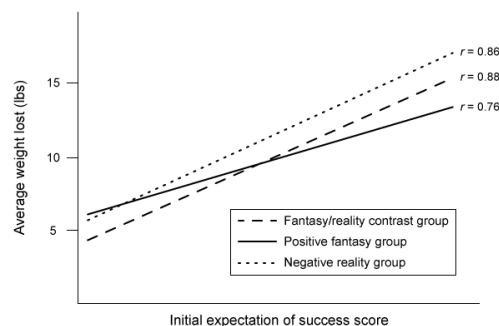


Figure 2 Initial expectation of success score versus average weight loss

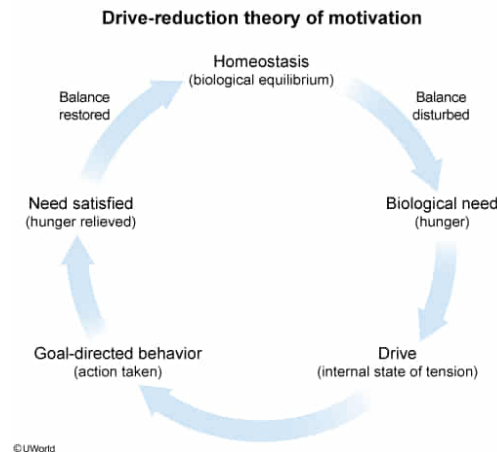
Which of the following proactive weight loss strategies is most consistent with drive-reduction theory?

- ☐ A. Keeping a daily food journal to record all calories consumed [4%]
- ☐ B. Using new clothes as a weekly reward for weight loss [13%]
- ☐ C. Engaging in cardiovascular and strength-training exercises [9%]
- ☒ D. Eating food more slowly and mindfully, and stopping when full [72%]

Correct

71% answered correctly

Explanation:



Drive-reduction theory proposes that motivation is a result of a **disruption of homeostasis**, which generates a **biological need**. The biological need generates a **drive** to fulfill that need, which prompts action. For example, when blood glucose levels drop, an organism feels hungry and engages in food-seeking behaviors. Therefore, eating slowly and mindfully and stopping when full is a strategy that addresses a physiological need and is most consistent with drive-reduction theory.

(Choice A) Keeping a food journal does not directly address the reduction of a physiological drive.

(Choice B) Secondary reinforcers (eg, new clothes) do not address the reduction of a drive; this strategy more closely aligns with incentive theory.

(Choice C) Exercise disrupts homeostasis. Although this disruption is positive and leads to healthy physiological adaptations, this strategy is the opposite of drive-reduction theory.

Educational objective:

According to drive-reduction theory, motivation is the result of a disruption in homeostasis, which produces a physiological need (eg, hunger), which leads to a drive (eg, desire to eat). Homeostasis is reestablished once the drive is reduced.

Time spent: 0 sec
Subject:
Behavioral Sciences

QID: 400510
System:
Motivation, Emotion, Attitudes, Personality, and Stress

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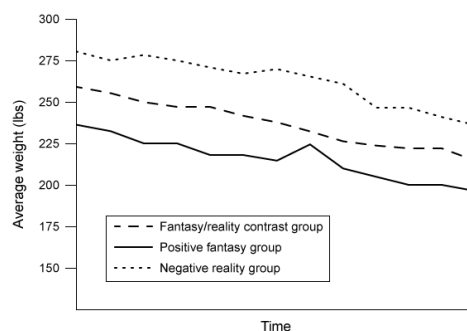


Figure 1 Average weight recorded for each group during three-month follow-up

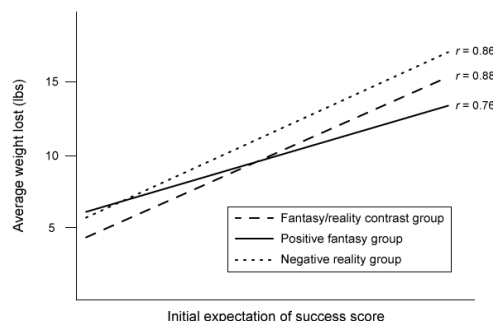


Figure 2 Initial expectation of success score versus average weight loss

If the study researchers concluded that their hypothesis was correct, which of the following was most likely their hypothesis?

- ☒ A. Positive thinking, as measured by expectation of success scores, will be positively correlated with success at attempted weight loss. [68%]
- ☐ B. Negative thinking, reflecting on the negative aspects that could impede weight loss, will be negatively correlated with success at attempted weight loss. [9%]

- ☐ C. The positive fantasy condition will lose significantly more weight than the fantasy/reality contrast condition and the negative reality condition. [8%]
- ☐ D. Fantasizing about the positive aspects of weight loss will be more effective than thinking about the reality of losing weight, regardless of the expectation of success. [13%]

Correct

68% answered correctly

Explanation:

Figure 2 shows the correlation between weight loss and the initial expectation of success scores of participants in each group. In all groups, individuals with *lower expectation of success scores lost less weight*, and those with *higher expectation of success scores lost more weight*. Therefore, expectation of success was positively correlated with weight loss, or, as expectation of success scores increased, so did the amount of weight loss.

Pearson's **correlation coefficient (r)** indicates the strength and direction of the linear relationship between two variables. The **closer r** is to **-1** or **1** (ie, an absolute value of 1), the **stronger the correlation**. Positive r -values reflect a positive correlation between variables (as one variable increases, so does the other) and negative r -values indicate a negative correlation between variables (as one variable increases, the other decreases).

(Choice B) The negative reality group lost weight over time (negative thinking was associated with successful weight loss); therefore, negative thinking does not appear to be negatively correlated with weight loss.

(Choices C and D) Figure 1 shows that each group had a similar difference between the average starting weight and average ending weight, suggesting little difference in weight loss success among the groups. The positive fantasy group appeared to have lost approximately the same amount of weight as the other two groups, although the strength of the correlation between success scores and weight loss is the weakest, reflected in the correlation coefficient that has the lowest absolute value.

Educational objective:

A correlation coefficient (r -value) with an absolute value close to 1 indicates a strong linear relationship between variables. Positive correlations reflect scores that move in the same direction (both increase; both decrease), while negative correlations reflect scores that move in opposite directions (one increases while the other decreases).

Time spent:0 sec

Subject:

Behavioral Sciences

QID:400511

System:

Motivation, Emotion, Attitudes, Personality, and Stress

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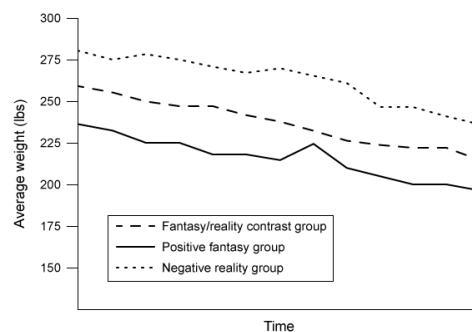


Figure 1 Average weight recorded for each group during three-month follow-up

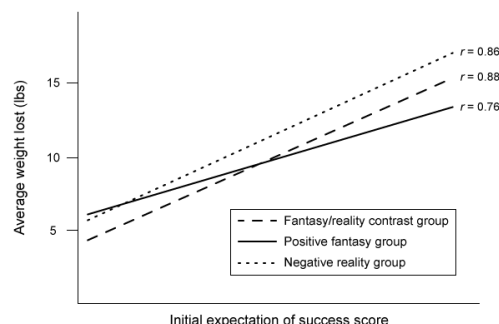


Figure 2 Initial expectation of success score versus average weight loss

Which of the following participants are most likely to experience learned helplessness?

- ☐ A. Those who selected "not successful" and "complete control." [3%]
- ☐ B. Those who selected "very successful" and "no control." [3%]
- ☐ C. Those who selected "very successful" and "complete control." [1%]
- ☒ D. Those who selected "not successful" and "no control." [91%]

Correct

91% answered correctly

Explanation:

		Self-efficacy	
		High (I am good at losing weight)	Low (I am terrible at losing weight)
Locus of control	Internal (I am in control of my weight loss)	"If I try hard, I can stick to my diet. I am in control of my exercise."	"I'm no good at dieting and exercise. It's totally my fault that I can't lose weight."
	External (My weight loss is out of my control)	"I am sticking to my diet but I'm on this medication that causes me to gain weight."	"No matter how hard I try, I'm no good at this and the world is against me." (learned helplessness)

Learned helplessness describes feelings of extreme **powerlessness** along with a perceived **lack of control**, often resulting from a series of failures or trauma. When an individual starts to feel that his efforts are futile and stops trying altogether, learned helplessness has occurred. Learned helplessness has been associated with an external locus of control and low self-efficacy.

Locus of control is defined by whether an individual believes her actions will **impact** the **outcome** of a situation. An individual with an *internal locus of control* feels that she has power to influence the outcome; an individual with an *external locus of control* feels that the outcome is due to luck or fate, which she cannot influence.

Self-efficacy describes a person's beliefs about his own **competence** or **ability** at a certain task. Self-efficacy can vary from task to task. For example, someone with high self-efficacy for math may have low self-efficacy for playing soccer. Self-efficacy can also change over time. For example, after attending a soccer camp, the individual's self-efficacy for soccer may be high.

The expectation of success score reflects self-efficacy, whereas the control of success score reflects locus of control. Those who selected "not successful" (low self-efficacy for weight loss) and "no control" (external locus of control for weight loss) are the *most likely* to experience learned helplessness (**Choices A, B, and C**).

Educational objective:

Learned helplessness describes feelings of powerlessness and loss of control, often occurring after a series of failures or trauma. Learned helplessness is tied to low self-efficacy (personal beliefs about one's abilities) and an external locus of control (personal beliefs about whether one can influence the outcome of a situation).

Time spent:0 sec
Subject:
Behavioral Sciences

QID:400512
System:
Motivation, Emotion, Attitudes, Personality, and Stress